INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Olaparib · adjuvant breast treatment

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Regimen source
NCCP 00588
Version 5b

Continuous daily treatment

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Reduce the chance of invasive recurrence after standard chemotherapy and local treatment. It may be taken alongside prescribed endocrine therapy for hormone receptor-positive disease.

PARP INHIBITOR

Olaparib (Lynparza)

Disrupts deoxyribonucleic acid (the cell's genetic material) (DNA) repair in susceptible cancer cells

BRCA-related repair is impaired → Olaparib blocks and traps PARP → DNA damage accumulates in cancer cells

BRCA1 and BRCA2 help cells repair breaks in DNA. Some cancers with these changes rely more heavily on other repair routes. Olaparib inhibits PARP activity and traps PARP on DNA, making it harder for those cancer cells to survive DNA damage. Normal blood-forming cells can also be affected, which is why regular blood counts matter.

Keep the four-weekly blood reviews and report persistent nausea, marked fatigue, infections, bleeding or new respiratory symptoms. Severe blood abnormalities need review even if you feel well.

The exact treatment being discussed

Who this covers

Adults with a confirmed pathogenic germline BRCA1/2 mutation, HER2-negative high-risk early breast cancer and previous neoadjuvant or adjuvant chemotherapy. The high-risk definition depends on hormone receptors, prior chemotherapy setting, surgical pathology and, for one pathway, CPS+EG score.

NCCP 00588 v5b · official regimen · EMA Lynparza product information

Dose and administration

Usually 300 mg twice daily; follow the individual prescription. Swallow tablets whole with water, with or without food. Do not chew, crush, dissolve or divide them. If a dose is missed, take the next normal dose at its usual time; do not double it. Check tablet strength after any dose change.

Duration and decisions

For breast indication 00588 h: up to one year, or earlier if the cancer recurs or side effects make treatment unsuitable. This is different from olaparib durations in ovarian, prostate and pancreatic cancer.

Individual safety and source differences

This patient suite is only for breast adjuvant indication 00588 h. Kidney/liver impairment, previous dose reductions and interacting medicines can change the dose. New blood abnormalities can occasionally signal a serious marrow problem and must be investigated.

Why this treatment may be chosen

In people with the high-risk inherited BRCA-associated breast cancer studied, one year of olaparib reduced invasive recurrence and improved survival. Longer follow-up shows that the benefit persists after treatment ends. Eligibility is specific, and benefit for an individual remains uncertain.

Evidence is specific to the population and schedule studied. Discuss expected benefit, alternatives and uncertainty for your own treatment.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Contact the oncology team urgently for fever or feeling acutely unwell, unusual bleeding, new or worsening breathlessness/cough, severe weakness, persistent vomiting or inability to drink. Chest pain, severe breathing difficulty or collapse require emergency help.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Reference timing, not booked appointments. Ask who requests each test, when it is due, who reviews the result and whom to contact if it is missing. Delays can change cycle numbers; time-based monitoring remains time-based.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00588 v5b · official regimen · EMA Lynparza product information · Pan-Asian adapted ESMO early breast cancer guidelines · Pan-Asian adapted ESMO metastatic breast cancer guidelines · ESO-ESMO ABC 5 consensus: tumour-marker interpretation

THE WHOLE TREATMENT JOURNEY

Olaparib · adjuvant breast treatment: treatment and checks

Your treatment pattern

Continuous daily treatment

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Germline BRCA1/2-associated, HER2-negative high-risk early breast cancer

Take olaparib tablets morning and evening every day. There is no planned week off. Blood-review appointments are usually every four weeks during this one-year breast course.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Treatment plan NCCP 00588	Confirm indication, goal and current prescription	Continuous tablets through a planned one-year breast course	Continue according to response, tolerance and prescribed course	Team review before restart or dose change	Agree completion or next treatment
Blood count, kidneys and liver NCCP 00588 · SMPC	Baseline full blood count (FBC), renal and liver profile; recover previous blood toxicity before starting	First repeat around four weeks; earlier if symptoms or results require	Every four weeks	Every four weeks through the twelve-month course	Recheck abnormalities; severe/ persistent blood toxicity may need marrow investigation

Care / check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Symptoms, medicines and tablet supply NCCP 00588 · SMPC	Review previous treatment, medicines, supplements and anti-sickness plan	Check nausea, intake, fatigue and adherence; do not wait if unwell	Review at blood/clinic contacts; check new medicines	Continue reviews and arrange supplies	Review recovery and the final olaparib stop plan
Surgery, radiation and hormone treatment NCCP 00588 · SMPC	Complete the agreed local treatment; confirm timing after radiation	Olaparib begins after local-treatment recovery; hormone treatment may overlap	Continue separately prescribed endocrine therapy where indicated	Endocrine therapy can continue alongside olaparib	Olaparib finishes by the planned year; endocrine treatment may continue
Breast surveillance and scans NCCP 00588 · SMPC	Confirm surgical follow-up and breast-imaging plan	No automatic olaparib-specific computed tomography (CT) scan	Breast follow-up and symptom-directed investigation	Annual mammography of remaining breast tissue; symptom-directed tests. No routine asymptomatic CT or tumour markers.	Continue surveillance after olaparib; investigate new symptoms promptly
Echo, electrocardiogram (ECG) and bone density NCCP 00588 · SMPC	Check if previous/other treatments or existing conditions require testing	No routine olaparib-specific echo or ECG	Keep any separate endocrine or cardiac monitoring plan	dual-energy X-ray absorptiometry (bone density) (DXA) follows endocrine treatment/ fracture risk, not the olaparib calendar	Continue only as indicated by the relevant treatment and risk plan
Pregnancy and contraception NCCP 00588 · SMPC	Pregnancy exclusion/test where relevant; contraception discussion	Follow agreed contraception	Consider repeat testing where indicated	Continue contraception throughout	Women: six months after; men/partners: three months. No breastfeeding for one month after

What each check is for

1



Treatment and safety review

Before each planned treatment or four-week review for olaparib

FBC, renal and liver profile every 4 weeks throughout the 12-month breast course; earlier/additional tests for symptoms or abnormal results. Review adherence, tablet supply, nausea, fatigue and all medicines.

Ask / act: The treating team confirms results and the plan; this map does not authorise a dose.

2

Benefit and continuation

At agreed clinical review points

In people with the high-risk inherited BRCA-associated breast cancer studied, one year of olaparib reduced invasive recurrence and improved survival. Longer follow-up shows that the benefit persists after treatment ends. Eligibility is specific, and benefit for an individual remains uncertain.

Ask / act: Discuss benefit, toxicity and priorities before continuing or changing treatment.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks throughout treatment

Blood tests assess treatment safety. Scans assess disease or investigate symptoms. Echo and ECG answer different questions.

Before treatment

Confirm the germline BRCA result and breast eligibility. Review blood count, kidney and liver tests, recovery from previous treatment, medicines and supplements, and pregnancy testing where relevant. Confirm the surgery/radiotherapy sequence and the planned start.

During treatment

NCCP specifies FBC, kidney and liver tests every four weeks for the first twelve months. Reviews also assess nausea, appetite, energy, bruising, infections and breathing symptoms. Extra tests or an interruption may be needed if results or symptoms change.

Heart checks

Olaparib alone has no routine ECG or echo surveillance timetable in this NCCP regimen. Heart checks may still be needed because of previous anthracycline/HER2 treatment, existing disease or new symptoms.

Bone health

There is no olaparib-specific DXA schedule. If an aromatase inhibitor or ovarian suppression is also prescribed, retain that treatment's separate bone-density and bone-health plan.

Your wider treatment plan

Additional treatments depend on the confirmed setting and indication.

Early-breast follow-up

After early breast cancer treatment, annual mammography is generally used for remaining breast tissue: both breasts after conservation, or the opposite breast after unilateral mastectomy. Routine mammography of the mastectomy side is not required. Routine CT, bone scans and tumour markers are not recommended solely to look for recurrence in an asymptomatic person. Safety blood tests for ongoing olaparib are a separate requirement. New symptoms need assessment between appointments.

Tumour markers

No routine tumour-marker surveillance for asymptomatic early breast cancer.

Pregnancy, fertility and contraception

Olaparib can harm an unborn baby. Discuss effective contraception before treatment; EMA recommends two reliable complementary methods for women during treatment and for six months afterwards. Men and their partners need contraception during treatment and for three months afterwards. Do not breastfeed during treatment or for one month after the last dose.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Daily treatment and review milestones

No dates or personal details are needed. Treatment is continuous unless the prescriber changes it; there is no invented chemotherapy rest week.

Treatment or check	Timing in this view
Before the first dose	**Before starting** Eligibility, bloods and medicines review
Daily treatment	**First cycle / month → Early treatment → Ongoing treatment** Usually 300 mg twice daily; follow the individual prescription
Safety monitoring	**First cycle / month → Early treatment → Ongoing treatment → Change / completion** Keep the blood-test and symptom-review plan
Completion / next decision	**Change / completion** Up to one year in this breast indication

Swallow tablets whole with water, with or without food. Do not chew, crush, dissolve or divide them. If a dose is missed, take the next normal dose at its usual time; do not double it. Check tablet strength after any dose change.

Symptoms can occur at any time. Milestone columns show order, not equal lengths of time.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Evidence depends on the population

Different studies answer different questions; do not compare their results as if populations and follow-up were identical.

OlympiA: four-year survival analysis

The earlier survival update showed that the benefit extended beyond preventing invasive recurrence. These results apply to the selected high-risk inherited BRCA-associated early breast population, not every HER2-negative breast cancer.

OlympiA: four-year survival analysis

OlympiA: sustained benefit at six years

Longer follow-up confirms that reductions in invasive recurrence and improvements in survival persist after the planned year of treatment. This is follow-up after one year of olaparib, not a longer prescribed course; rare late marrow effects still need attention.

OlympiA: sustained benefit at six years

Benefit, course and continuing checks

Before and during treatment

FBC, renal and liver profile every 4 weeks throughout the 12-month breast course; earlier/additional tests for symptoms or abnormal results. Review adherence, tablet supply, nausea, fatigue and all medicines.

Heart checks and the full course

No routine olaparib-specific ECG, echo or DXA; follow separate previous/ongoing treatment risks. Breast surveillance continues after olaparib, without automatic asymptomatic CT or tumour-marker scheduling. High risk:TNBC residual disease after neoadjuvant therapy or node-positive/≥2 cm after adjuvant chemotherapy; HR-positive disease requires residual disease with CPS+EG≥3 after neoadjuvant therapy or≥4 positive nodes after adjuvant chemotherapy. Ideally start within 12 weeks of the last surgery, chemotherapy or radiation.

Practical support

Nausea support matched to oral treatment

NCCP classifies oral olaparib as moderate-to-high emetic risk. Agree an anti-sickness plan under local policy, and seek review if oral intake falls. The SmPC describes early/intermittent symptoms; do not automatically copy an intravenous chemotherapy antiemetic cycle.

Check the antiemetic interaction

Aprepitant is a moderate CYP3A inhibitor in NCCP 00588 and can increase olaparib exposure. The prescriber/pharmacist should choose and reconcile supportive medicines; never add an antiemetic from another regimen without checking.

Blood and infection support

Anaemia or other low counts can lead to interruption, dose adjustment, transfusion or additional investigation. There is no universal routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) course for every person on this regimen.

Medicines and supplements

Review all prescriptions, over-the-counter drugs and herbal products. Strong/moderate CYP3A inhibitors or inducers can change exposure. Avoid grapefruit products and seek advice before starting anything new.

Stomach symptoms

NCCP allows consideration of a proton pump inhibitor under local policy. This is symptom- and patient-dependent, rather than a compulsory medication for everyone.

Review the full medicine plan

NCCP classifies oral olaparib as moderate-to-high emetic risk. Agree an anti-sickness plan under local policy, and seek review if oral intake falls. The SmPC describes early/intermittent symptoms; do not automatically copy an intravenous chemotherapy antiemetic cycle. Aprepitant is a moderate CYP3A inhibitor in NCCP 00588 and can increase olaparib exposure. The prescriber/pharmacist should choose and reconcile supportive medicines; never add an antiemetic from another regimen without checking. Anaemia or other low counts can lead to interruption, dose adjustment, transfusion or additional investigation. There is no universal routine G-CSF course for every person on this regimen. Review all prescriptions, over-the-counter drugs and herbal products. Strong/moderate CYP3A inhibitors or inducers can change exposure. Avoid grapefruit products and seek advice before starting anything new. NCCP allows consideration of a proton pump inhibitor under local policy. This is symptom- and patient-dependent, rather than a compulsory medication for everyone. Agree an oral antiemetic prevention / rescue plan before treatment. Continuous oral treatment does not use the Day 1–4 intravenous (IV) chemotherapy template. Confirm agent, dose, frequency, duration and interactions; no default daily steroid order.

Taking tablets / attending treatment

Practical treatment details

Make treatment fit into daily life

Tablets are not interchangeable with capsules.

After chemotherapy and local treatment

In OlympiA, surgery and any indicated radiotherapy were completed before olaparib, with at least a two-week gap after local treatment.

A daily routine at home

Check the strength and number of tablets on each new prescription.

Keep a sufficient supply before weekends or holidays.

No infusion pump or port is needed for olaparib itself.

After the planned year

Report persistent unexplained fatigue, infections, bruising or breathlessness even after olaparib finishes.

Questions to ask

When do I take or receive treatment?

Take olaparib tablets morning and evening every day. There is no planned week off. Blood-review appointments are usually every four weeks during this one-year breast course. Swallow tablets whole with water, with or without food. Do not chew, crush, dissolve or divide them. If a dose is missed, take the next normal dose at its usual time; do not double it. Check tablet strength after any dose change.

When are bloods, ECGs or echoes needed?

Confirm the germline BRCA result and breast eligibility. Review blood count, kidney and liver tests, recovery from previous treatment, medicines and supplements, and pregnancy testing where relevant. Confirm the surgery/radiotherapy sequence and the planned start. NCCP specifies FBC, kidney and liver tests every four weeks for the first twelve months. Reviews also assess nausea, appetite, energy, bruising, infections and breathing symptoms. Extra tests or an interruption may be needed if results or

symptoms change. Olaparib alone has no routine ECG or echo surveillance timetable in this NCCP regimen. Heart checks may still be needed because of previous anthracycline/HER2 treatment, existing disease or new symptoms.

What should make me call today?

Contact the oncology team urgently for fever or feeling acutely unwell, unusual bleeding, new or worsening breathlessness/cough, severe weakness, persistent vomiting or inability to drink. Chest pain, severe breathing difficulty or collapse require emergency help.

Explain the plan back: Which medicine is taken or given when, what checks are expected next, and which symptoms mean you should call today?

Breast adjuvant treatment only · NCCP00588h. These materials cover olaparib after treatment for high-risk early breast cancer. Other cancers use different eligibility, duration and follow-up; use the matching NCCP indication and your treating team's plan.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Olaparib · adjuvant breast treatment · reviewed supportive care

Active medicines: Olaparib (Lynparza)

Emetogenic risk: Oral moderate–high

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- NCCP classifies oral olaparib as moderate-to-high emetic risk. Agree an anti-sickness plan under local policy, and seek review if oral intake falls. The SmPC describes early/intermittent symptoms; do not automatically copy an intravenous chemotherapy antiemetic cycle.
- Aprepitant is a moderate CYP3A inhibitor in NCCP 00588 and can increase olaparib exposure. The prescriber/pharmacist should choose and reconcile supportive medicines; never add an antiemetic from another regimen without checking.
- Anaemia or other low counts can lead to interruption, dose adjustment, transfusion or additional investigation. There is no universal routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) course for every person on this regimen.
- Review all prescriptions, over-the-counter drugs and herbal products. Strong/moderate CYP3A inhibitors or inducers can change exposure. Avoid grapefruit products and seek advice before starting anything new.
- NCCP allows consideration of a proton pump inhibitor under local policy. This is symptom- and patient-dependent, rather than a compulsory medication for everyone.
- Agree an oral antiemetic prevention / rescue plan before treatment. Continuous oral treatment does not use the Day 1–4 intravenous (IV) chemotherapy template. Confirm agent, dose, frequency, duration and interactions; no default daily steroid order.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Breast adjuvant treatment only · National Cancer Control Programme (NCCP) 00588h. These materials cover olaparib after treatment for high-risk early breast cancer. Other cancers use different eligibility, duration and follow-up; use the matching NCCP indication and your treating team's plan.

NCCP 00588 · VERSION 5B

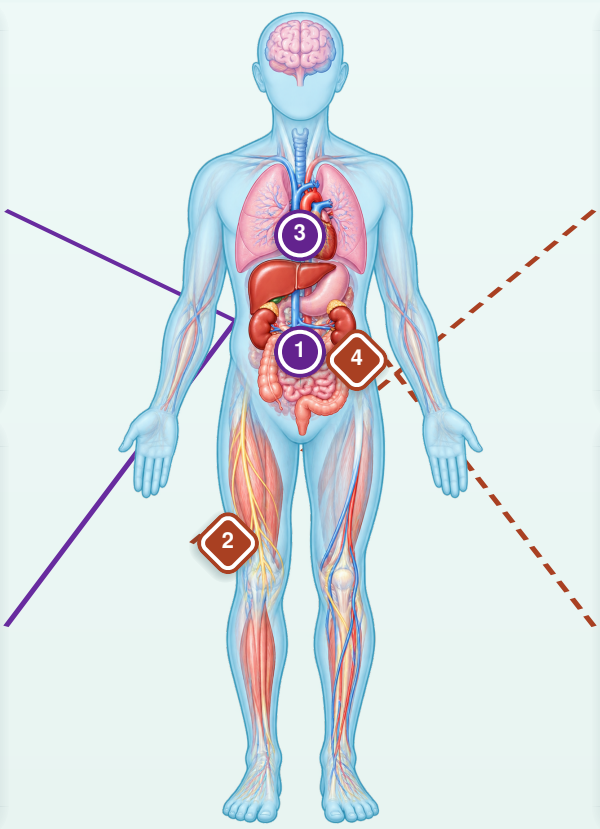
Olaparib · adjuvant breast treatment

Take olaparib tablets morning and evening every day. There is no planned week off. Blood-review appointments are usually every four weeks during this one-year breast course.

Oral targeted therapy · PARP inhibitor · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Nausea and appetite

Olaparib

Ask for help early if nausea or vomiting makes it difficult to eat, drink or take tablets.

2 Blood counts and energy

Olaparib

Anaemia can cause marked tiredness or breathlessness; low white cells and platelets can cause infection or bruising.

3 New breathing symptoms

Olaparib

A new cough or worsening breathlessness needs prompt assessment; uncommon lung inflammation is important to recognise.

4 Liver warning signs

Olaparib

Report jaundice, dark urine or persistent upper abdominal discomfort; liver tests help investigate possible injury.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team urgently for fever or feeling acutely unwell, unusual bleeding, new or worsening breathlessness/cough, severe weakness, persistent vomiting or inability to drink. Chest pain, severe breathing difficulty or collapse require emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Nausea / appetite changes	☐	☐	☐	
Fatigue / anaemia	☐	☐	☐	
Infections / bruising	☐	☐	☐	
New cough / breathlessness	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Contact the oncology team urgently for fever or feeling acutely unwell, unusual bleeding, new or worsening breathlessness/cough, severe weakness, persistent vomiting or inability to drink. Chest pain, severe breathing difficulty or collapse require emergency help. Emergency symptoms: 112 / 999.

Carry your treatment details

Breast adjuvant treatment only · NCCP 00588h. These materials cover olaparib after treatment for high-risk early breast cancer. Other cancers use different eligibility, duration and follow-up; use the matching NCCP indication and your treating team’s plan.

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Olaparib · adjuvant breast treatment

NCCP 00588 v5b

Take olaparib tablets morning and evening every day. There is no planned week off. Blood-review appointments are usually every four weeks during this one-year breast course.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Contact the oncology team urgently for fever or feeling acutely unwell, unusual bleeding, new or worsening breathlessness/cough, severe weakness, persistent vomiting or inability to drink. Chest pain, severe breathing difficulty or collapse require emergency help.

https://sactcheck.com/patient/00588/scope.html · no patient information encoded.

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00588 v5b · official regimen**
   https://healthservice.hse.ie/documents/6397/olaparib_tablet_monotherapy_588.pdf
2. **EMA Lynparza product information**
   https://www.ema.europa.eu/en/documents/product-information/lynparza-epar-product-information_en.pdf
3. **Pan-Asian adapted ESMO early breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC11145753/
4. **Pan-Asian adapted ESMO metastatic breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC10186487/
5. **ESO-ESMO ABC 5 consensus: tumour-marker interpretation**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC7510449/
6. **Garber et al. OlympiA six-year update; Ann Oncol 2026; DOI 10.1016/j.annonc.2026.08.002**
   https://www.annalsofoncology.org/article/S0923-7534%2826%2901456-0/fulltext
7. **OlympiA: adjuvant olaparib primary analysis**
   https://www.nejm.org/doi/full/10.1056/NEJMoa2105215
8. **OlympiA: four-year survival analysis**
   https://pubmed.ncbi.nlm.nih.gov/36228963/
9. **OlympiA: sustained benefit at six years**
   https://doi.org/10.1016/j.annonc.2026.08.002
10. **NCCP antiemetic guidance; reconcile with regimen and individual prescription**
    https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf